



HMIS 033b:

HEALTH UNIT WEEKLY EPIDEMIOLOGICAL SURVEILLANCE REPORT

DESCRIPTION AND INSTRUCTIONS

- Objective:** To report cases of notifiable diseases, other diseases of public health importance, OPD attendances, eMTCT appointments, Malaria tested and treated, TB tested and treated and stock balances of select tracer items.
- Frequency:** Every Monday of the Week in a calendar year
- Copies:** **Three Copies (Triplicate).** Triplicate stays at the health unit, duplicate is sent to the Health-Sub-District Headquarters, the Original copy is sent to the District health office.
- Responsibility:** Health Unit In-charge

PROCEDURE:

1. All health units must report this information (Government, Private Health Providers and PNFP) to the HSD and DHO. In addition, the health facility should report any other priority disease and/or events as required by the District Health Officer.
2. The report should be clearly labeled to show the period covered i.e. date for the first (Monday) and last day (Sunday) of the week for which the report is being made.
3. For each disease category indicate the number of new cases during the week (cases this week), the number of deaths that occurred during the week (deaths this week) and those tested (tested cases), tested positive (Pos(+ve) cases)
4. For Maternal deaths, Perinatal deaths, OPD and eMTCT summary, summary of malaria cases tested and treated, summary of tuberculosis cases tested and treated during the week, tracer medicines - stock balance, HIV testing kits, TB, & eMTCT drugs - stock balance, summary of Genexpert report and summary of IPT initiation, refer to the SOPs for the case definitions and how to extract the data.
5. For Other conditions (EPC.), refer to the given table below of other priority diseases/conditions for the codes when reporting using mTrac.
6. Refer to the SOPs for the case definitions and how to extract the data.
7. The health unit continues to report every week throughout the year whether there are cases or not and this should take care of "zero" report.
8. Transcribe the data every week into HMIS form 033c (Health Unit Weekly Epidemiological Surveillance Summary for the year) for the respective weeks. For example 10 cases with 2 deaths are recorded as 10 (2).



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Health Facility Name.....Week..... Month Calendar Year:

SELECTED OTHER PRIORITY DISEASES UNDER IDSR (INTEGRATED DISEASE SURVEILLANCE AND RESPONSES)

Epidemic Prone Diseases/ Conditions	Code	Diseases/conditions targeted for elimina- tion or eradication	Code	Disease of public health importance	Code
Chikungunya	CG.	Dracunculiasis	DC.	Diarrhoea with dehydration <5	DD.
Dengue	DG.	Onchocerciasis	OC.	Severe pneumonia <5	PN.
Influenza-like illness	IL.	Buruli ulcer	BU.	Human African Trypanosomiasis	TX.
Acute viral hepatitis	HP.	Lymphatic Filariasis	LF.	Trachoma	TR.
		Noma	NO.	Schistosomiasis	SC.
		Human influenza due to a new subtype	HN.	Diphtheria	DP.
		Severe Acute Respiratory Syndrome (SARS)	SS.	Pertussis (Whooping cough)	WC.
		Smallpox	SP.	Brucellosis	BC.
				Kala azar	KA.
				Nodding Syndrome	NS.
				Adverse Drug Reactions (ADR)	AR.

Note: 1. The health unit continues to report every week throughout the year whether there are cases or not and this should take care of “**zero report**”.

2. For standard case definition of all priority IDSR diseases and conditions refer to the **standard case operating procedures**.



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Date of Report _____ Period: From (Date) 1 _____ To (Date) _____

Week No. (#) _____ Health Facility _____ Code _____ Parish _____

Sub-county _____ HSD _____ District _____

CASES AND DEATHS THIS WEEK

1. CASES.		Code	Total Cases this week	2. DEATH.	Tested Cases	Pos(+ve) cases
				Total Death this week		
1	Malaria (Confirmed)	MA.				
2	Dysentery	DY.				
3	Severe Acute Respiratory Infection (SARI)	SA.				
4	Acute Flaccid Paralysis	AF.				
5	Adverse Events Following Immunization (AEFI)	AE.				
6	Animal Bites (suspected rabies)	AB.				
7	Bacterial Meningitis	MG.				
8	Cholera	CH.				
9	Guinea Worm	GW.				
10	Measles	ME.				
11	Neonatal tetanus	NT.				
12	Plague	PL.				
13	Typhoid Fever	TF.				
14	Hepatitis B	HB.				
15	Rifampicin resistant TB cases	DR.				
16	Yellow Fever	YF.				
17	Other Viral Hemorrhagic Fevers (EVD, MVD, RVF, CCHF)	VF.				
18	Leprosy	LP.				
19	Anthrax	AX.				
20	Maternal death	MD.				
21	Macerated Still births	MB.				
22	Fresh Still Birth	FB.				
23	Early Neonatal deaths 0-7 days	ND.				
24	Covid-19	CV.				



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3. Other conditions (refer to the table below for the other priority diseases/conditions)

CASES (EPC.)		Code	Total Cases this week	DEATH (EPD.)	Tested Cases	Pos(+ve) cases
				Total Death this week		
1						
2						
3						
4						
5						
6						
7						
8						
9						
10						

4. OPD AND EMTCT SUMMARY

APT.						Tick upon Feedback
	OPD New Attendance	Total OPD Attendance	Total Death	Expected eMTCT Mothers on appt	eMTCT Missed appointments	
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5. SUMMARY OF MALARIA CASES TESTED AND TREATED

	Suspected malaria (fever)	Cases tested with RDT	RDT Positive Cases	Cases tested with Microscopy	Microscopy Positive Cases	Not tested cases treated	RDT Negative Cases Treated	RDT Positive Cases Treated	Microscopy Negative Cases Treated	Microscopy Positive Cases Treated
MAT.	.	.	.	.	.	.	.	.	.	.

Tick upon Feedback

6. SUMMARY OF TUBERCULOSIS CASES TESTED AND TREATED DURING THE WEEK

	Clients screened for TB at all entry points	Presumptive TB cases identified at all entry points	New and Relapse diagnosed and registered	New and Relapse TB cases started on treatment	Bacteriologically TB cases registered with Genexpert	Number of TB contacts traced and screened
TB.	.	.	.	.	.	.

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7. TRACER MEDICINES - STOCK BALANCE

	Artemether/Lumefantrine 20/120 mg tablet	ORS Sachets with zinc tablet	Measles -Rubella Vaccine vial	Amoxicillin Dispersible 250mg Tablets	Depot Medroxyprogesterone (DMPA)_ IM + SC	Artesunate 60 mg vial	Sulfadoxine/Pyrimethamine Tablet	Malaria Rapid Diagnostic tests
TRA.	.	.	.	.	.	.	.	.

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8. HIV TESTING KITS, TB, & eMTCT Drugs - STOCK BALANCE

	Determine HIV 1 & 2 screening test	Stat -pack HIV Confirmatory rapid tests	Abacavir/Lamivudine/Dolutegravir (ABC/3TC/DTG) 600/300/50mg	Nevirapine (NVP) 10mg/ml oral susp.	Tenofovir/Lamivudine/Dolutegravir (TDF/3TC/DTG) 300/300/50mg tablets	RHZ 75/50/150mg Blisters of 28 tablets	RHZE 150/75/400/275mg Blisters of 28 tablets	RH 150/75mg Blister of 28 tablets	RH 75/50mg Blister of 28 tablets	Rifapentine/Isoniazid 300/300mg
ARV.	.	.	.	.	.	.	.	.	.	.

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9. SUMMARY OF GENEXPERT REPORT FOR GENEXPERT / TRUENAT SITES ONLY

	No of samples tested	No of samples rejected	Total MTB detected	Total No. Rif R	No. of errors/invalid results	No. GeneXpert modules working	No of cartridges remaining
GP.	.	.	.	.	.	.	.

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10. SUMMARY OF TPT INITIATION

	Number of Adult ART clients initiated TPT	Number of children and adolescents on ART who started TPT	Number of children 0 – 4 yrs who are contacts of TB patients initiated on TPT	Number of children 5 yrs and above who are contacts of TB patients initiated on TPT
TPT.	.	.	.	.

Tick upon Feedback